

Mild Distance Blur and Heavy Screen Use

Evidence-based triage, likely causes, 30-day fix plan, optometrist checklist, and one-page action sheet

Prepared for Josh Parris | 8 June 2026

Bottom line: The pattern you described - being able to read small text at about 5 metres but with slightly reduced crispness - is most consistent with mild digital eye strain/dry eye/focusing fatigue or a small prescription change such as mild myopia or astigmatism. It is usually fixable, but because it is a change in vision, book an optometrist appointment rather than guessing.

This report is educational, not a diagnosis. Seek urgent care immediately for the red flags listed in Section 2.

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1. Safety triage

Book an optometrist check soon, ideally within 1-2 weeks. The issue is not automatically alarming, but a new or persistent change in vision should be checked. In Australia, you can book directly with an optometrist; a GP referral is not needed. Medicare may cover a bulk-billed optometry eye test if the optometrist bulk bills; under-65s are generally eligible for one bulk-billed eye test every 3 years, and over-65s yearly. [2,4]

Same-day / urgent red flags

Symptom pattern	What to do
Sudden loss of vision, sudden severe blur, or a black/curtain/shadow over vision	Emergency department or urgent eye care immediately.
Blur with weakness/numbness on one side, drooping face, slurred speech, dizziness, loss of balance, or confusion	Call 000 for possible stroke symptoms. [2]
Eye pain, red eye, halos around lights, nausea/vomiting, or swelling/redness around the eye - especially while on sertraline	Same-day medical/eye care. Sertraline can rarely trigger angle-closure glaucoma in susceptible people. Do not abruptly stop sertraline without medical advice. [8]
Flashing lights, new floaters, wavy lines, light sensitivity, eye injury, or red eye with severe headache/feeling unwell	Same-day doctor/optometrist/ED assessment. [3]
One-eye blur that is sudden, worsening, painful, or clearly different from the other eye	Same-day optometrist or urgent eye clinic.

For your described pattern - mild, distant text still readable, slightly blurry, no reported sudden loss - this sounds more like optometry territory than emergency territory. But the red flags override that.

2. Ranked likely causes

- **1. Digital eye strain / computer vision syndrome:** Very likely if blur is worse after long screen blocks and improves with rest. Symptoms include eye strain, blurred vision, dry/irritated eyes, headaches and discomfort. Optometry Australia notes that long computer use, uncorrected vision problems, poor viewing distance, posture, lighting and glare can contribute. [1]
- **2. Dry eye from reduced blinking:** Very likely if vision fluctuates, feels smeary, or improves after blinking/tears. Screen use reduces blink rate and can dry the ocular surface; dry eye can cause transient blur, irritation and headaches. [3,6,7]
- **3. Focusing fatigue / accommodative spasm:** Likely if distance is blurrier after close work, then sharpens after looking outside or resting. Prolonged near focus can tire the focusing system. EyeWiki notes that accommodation and vergence parameters should be checked in people with eye strain symptoms. [5]
- **4. Mild myopia or astigmatism:** Quite possible. Even small uncorrected short-sightedness, long-sightedness or astigmatism can cause computer-related symptoms and mild distance blur. Astigmatism can make small distant text look doubled, smeared or not crisp. [1,5]
- **5. Binocular vision / convergence problem:** Possible if you get headaches, eye pulling, double vision, difficulty changing focus, or symptoms after reading. This is not fixed by willpower; it needs optometric assessment and sometimes specific lenses or supervised vision therapy. [5,7]

- **6. Headache, migraine, fatigue, stress or sleep debt:** Possible. Healthdirect lists headaches/migraine and medicines as possible causes of blurred vision. Stress and fatigue can make visual symptoms feel sharper and reduce your ability to tolerate screens. [2]
- **7. Sertraline-related visual symptoms:** Possible but not the first assumption. Official prescribing information lists blurred vision as an infrequent adverse reaction and warns about rare angle-closure glaucoma symptoms: eye pain, changes in vision, swelling or redness around the eye. [8]
- **8. Less common but important causes:** Cataract, glaucoma, retinal disease, diabetes/blood glucose swings, inflammatory eye disease, neurological causes and others are less likely in your exact story but are why a proper eye exam matters. [2,5]

3. What is harmless vs what needs care

Likely harmless/reversible	Optometrist soon	GP check	Urgent/emergency
Blur appears after screens, fluctuates, improves with blinking/rest, no pain/redness/neurological symptoms.	Any new or ongoing blur; suspected prescription change; dry eye symptoms lasting more than a few weeks; headaches with eye strain; driving/night vision concerns.	Blur with fatigue, thirst/urination, diabetes concern, medication changes, frequent migraines, ongoing dizziness, systemic symptoms, or anxiety/panic making symptoms hard to interpret.	Sudden vision loss; one-sided neurological symptoms; severe eye pain/redness; flashes/floaters/curtain; eye injury; severe headache with visual change; halos/nausea.

4. The 30-day fix plan

Goal: reduce reversible strain/dryness while confirming whether you need glasses, dry-eye treatment, or further medical review. Do not use this plan to delay urgent care if red flags appear.

Week 0-1: Set the baseline and remove the obvious strain

Action	Exact instruction
Book optometrist	Book a comprehensive eye exam in the next 1-2 weeks. Ask when booking: "I have new mild distance blur and heavy screen use; can the optometrist check refraction, astigmatism, dry eye, eye pressure, retina, and binocular vision?"
Do one baseline test only twice daily	Morning before screens and evening after screens: read the same distant sign/text at about 5 metres with both eyes, left eye, right eye. Rate clarity 0-10, dryness 0-10, headache 0-10. Do not keep re-checking all day.
Screen distance	Set main monitor 50-70 cm away - roughly arm's length. Larger screens may need 70-90 cm if text is still comfortably readable. [1,5,7]
Screen height	Top of monitor at or just below eye level. Centre of screen slightly below eye level, so your gaze is slightly downward rather than upward. [5,6,7]
Text size	Use at least 12-14 pt equivalent. On Windows/browser: start with 125% scaling or browser zoom 110-125% if you are leaning forward.

Brightness/contrast	Match screen brightness to the room. If the screen looks like a lamp, lower it. If it looks grey/dull and you squint, raise it. Use dark text on light background for long reading if comfortable.
Glare	Move monitor so windows/lights are not reflected on the screen. Use side lighting rather than a bright light behind you or shining into your face. [1,5,7]
Breaks	Use a timer: every 20 minutes, look outside or across the room for 20 seconds. Every 60 minutes, stand up for 2-5 minutes. After 2 hours of continuous screen work, take 10-15 minutes away from the screen. [5,7]
Blink drill	At each 20-minute break: 10 slow full blinks. Then close eyes 2 seconds, open, close 2 seconds, gently squeeze 2 seconds; repeat 3 times. [5]
Drops if dry	If eyes feel dry/gritty/smeary, use preservative-free lubricating eye drops 2-4 times/day for 14 days. Avoid “redness reliever” drops unless an optometrist/doctor specifically recommends them.

Weeks 2-3: Follow results and tighten the routine

- If the optometrist finds myopia/astigmatism, wear the prescribed glasses exactly as directed for distance/computer use. Do not judge the prescription in the first hour only; adaptation can take days.
- If dry eye is diagnosed, follow the optometrist’s plan. This may include preservative-free lubricants, warm compresses, lid hygiene, treating meibomian gland dysfunction, or prescription treatment in more persistent cases. [3,5]
- If binocular/accommodative issues are found, ask for a specific plan: lens options, prism if appropriate, supervised vision therapy if indicated, and review timing. Avoid random “eye exercise” apps unless your optometrist has diagnosed the target problem.
- Keep phone use further from your face: 35-45 cm minimum, larger text, no prolonged tiny-text reading in bed.
- Get daylight/outdoor distance looking daily: 20-30 minutes, preferably morning or lunch. This is more about visual hygiene, nervous system regulation and sleep support than reversing adult blur.

Week 4: Review whether it is actually fixed

- If distance clarity is now stable and symptoms are mild: keep the screen hygiene routine and follow optometrist review advice.
- If blur is constant despite screen changes and optometry is normal: book a GP appointment. Ask about headaches/migraine, blood pressure, blood glucose/HbA1c if appropriate, medication effects, sleep and sinus/neurological symptoms.
- If symptoms are worse, one-sided, painful, associated with flashes/floaters, or neurologically unusual: escalate urgently rather than waiting for the 30 days.

5. Simple daily protocol

Time	Protocol
Morning	Before heavy screens, look outside for 2 minutes. Notice whether distance vision is crisp before work. Drink water. If eyes are dry on waking, use preservative-free lubricating drops.

Start of work block	Set screen at arm's length, top just below eye level, text zoomed so you are not leaning forward, glare removed. Start a 20-minute repeating timer.
Every 20 minutes	Look at something at least 6 metres away for 20 seconds. Do 10 slow full blinks. Relax jaw, shoulders and forehead.
Every hour	Stand for 2-5 minutes. Look outside or down a hallway. Move neck/shoulders. This helps eyes and headaches.
Lunch	Get outside for 10-20 minutes if possible. Let your eyes focus at distance. Avoid eating while scrolling tiny phone text.
Afternoon	Use the same break pattern. If blur starts, pause for 5 minutes rather than pushing harder and squinting.
Evening	Reduce small-phone reading. Use bigger text and warmer/dimmer light. For sleep and nervous system regulation, stop optional screens 60 minutes before bed where possible.
Before bed	If eyes are dry/gritty, use lubricating drops or follow the optometrist's dry-eye plan. Do not do repeated vision checks in bed.

6. Optometrist checklist

Take this wording with you:

“My distance vision used to feel 20/20. I can still read small text at about 5 metres, but it is slightly blurry/not crisp, especially with heavy screen use. I also get headaches and take sertraline. Please check whether this is refractive error, dry eye, accommodative/binocular strain, or something else.”

Check	Why it matters
Visual acuity	Measure each eye separately and together for distance and near.
Refraction	Check for small myopia, hyperopia or astigmatism. Ask if a trial lens makes distant text crisp again.
Astigmatism detail	Ask whether mild cylinder correction is worth using for computer/distance tasks if symptoms match.
Accommodation	Check focusing flexibility/facility if blur appears after close work and improves after distance rest.
Binocular vision/convergence	Check eye teaming, near point of convergence, phoria/tropia, vergence reserves and whether symptoms fit convergence insufficiency/excess.
Dry eye and lids	Check tear film, tear breakup time, meibomian glands, eyelids, blepharitis and ocular surface staining where appropriate.
Eye pressure and anterior eye exam	Check intraocular pressure, pupil response, cornea, anterior chamber angle risk if indicated, and signs of inflammation.
Retina/optic nerve	Retinal exam, optic nerve assessment, and dilation/OCT/fundus photo if clinically indicated.
Medication/headache context	Mention sertraline, headaches, anxiety/stress, and any recent dose changes. Ask whether anything needs GP follow-up.
Glasses plan	Ask: “Do I need distance glasses, computer glasses, both, or no glasses? When should I wear them?”

Questions to ask before leaving

- What is my visual acuity in each eye?
- Do I have myopia, astigmatism, hyperopia or presbyopia beginning?
- Is this likely dry eye, focusing fatigue, binocular strain, or something else?
- Are my eye pressure, retina and optic nerve normal?
- Do I need drops, glasses, computer glasses, exercises/therapy, GP review, or just routine care?
- What symptoms should make me seek urgent care?
- When should I come back if it is not better?

7. Decision tree

If...	Most likely meaning	Action
Blur improves after 10-20 minutes away from screens	Digital eye strain, dry eye or focusing fatigue is likely.	Use the 30-day plan and still book optometrist if it keeps recurring.
Blur is constant even when rested	Small prescription change or another eye issue becomes more likely.	Optometrist within 1-2 weeks.
One eye is clearly blurrier than the other	Could still be refractive, but one-eye changes deserve more caution.	Prompt optometrist. Same day if sudden/painful.
Flashes, new floaters, curtain/shadow, or sudden loss	Possible retinal/urgent eye issue.	Emergency/same-day eye care.
Eye pain, red eye, halos, nausea, swelling/redness around eye	Possible acute eye inflammation/pressure issue; sertraline warning overlaps with this pattern.	Same-day medical/eye care. Do not abruptly stop sertraline without advice. [8]
Blur with weakness/numbness, drooping face, slurred speech, severe dizziness or confusion	Possible stroke or neurological emergency.	Call 000. [2]
Blur mainly with headache or aura	Migraine/fatigue possible, but new patterns should be checked.	Optometrist plus GP if new, severe, changing, or frequent.
Blur and dryness persist after several weeks of self-care	Dry eye or lid gland dysfunction may need targeted treatment.	Optometrist/GP review. [3]

8. What to avoid and what not to overbuy

- **Do not rely on blue-light glasses as the main fix.** A Cochrane review found blue-light filtering lenses may not reduce short-term eyestrain from computer work compared with non-blue-light lenses; evidence for sleep benefits is uncertain. Breaks, proper setup and dry-eye management matter more. [10]
- **Do not use redness-reliever drops as routine eye comfort drops.** They can mask irritation and are not the same as lubricating/artificial tears. Use preservative-free lubricating drops if dryness is the symptom, and ask an optometrist if symptoms persist.
- **Do not keep testing your vision all day.** Repeated checking can feed anxiety and make normal fluctuation feel threatening. Baseline twice daily is enough unless symptoms suddenly change.
- **Do not do random “eye strengthening” programs to reverse myopia.** Exercises do not reliably reverse refractive error. Vision therapy can help specific diagnosed binocular/accommodative problems, but it should be targeted by an optometrist.

- **Do not push through blurred vision by squinting harder.** Squinting may briefly sharpen text but worsens tension and headaches. Increase text size, take a break or use the correct glasses.

9. Tracking sheet for 14 days

Use this to give the optometrist useful information without obsessing.

Day	Morning clarity 0-10	Evening clarity 0-10	Dryness 0-10	Headache 0-10	Screen hours / notes
1					
2					
3					
4					
5					
6					
7					
8					
9					
10					
11					
12					
13					
14					

10. One-page printable action plan

Daily non-negotiables for the next 30 days

1. Book optometrist check: new mild blur + heavy screens + headaches + sertraline context.
2. Monitor at arm's length: 50-70 cm. Top of screen at or just below eye level.
3. Zoom text so you sit back: browser 110-125% or Windows scaling 125% if needed.
4. Remove glare: no window/lamp reflection on screen; match screen brightness to room.
5. Every 20 minutes: look 6 metres away for 20 seconds + 10 slow full blinks.
6. Every hour: stand and look outside/down hallway for 2-5 minutes.
7. If dry/smeary: preservative-free lubricating drops 2-4 times/day for 14 days.
8. Phone: hold 35-45 cm away, larger text, avoid tiny-text scrolling in bed.
9. Daylight: 20-30 minutes outside or looking into distance daily.
10. Sleep: stop optional screens 60 minutes before bed where possible.

Escalate urgently if: sudden vision loss, one-eye sudden blur, flashes/floaters/curtain, eye pain/redness/halos/nausea, severe headache with visual change, or stroke-like symptoms. Call 000 for stroke-like symptoms.

References

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